

Evaluating Feasibility and Adaptability of Stronger Together Peer Mentoring Program for Cancer Patients in Viet Nam Using the Consolidated Framework for Implementation Research

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Background

- Peer support — i.e., psychosocial support from those with shared lived experiences of cancer—is increasingly recognized as a valuable component of cancer care, and should be tailored to the resources and cultures in which they are being delivered.¹
- Few studies have evaluated cancer peer support programs using an implementation science lens in low-resource settings such as Viet Nam (VN).²
- Despite a high rate of delays and non-completion in cancer diagnosis and treatment, psychological distress in VN is widely overlooked or underestimated by local professionals.
- We used the Consolidated Framework for Implementation Research (CFIR)³ to analyze implementation determinants that affected the acceptability and feasibility of a pilot study of the Stronger Together (ST) peer mentoring model among breast and gynecologic cancer patients at four oncology centers/hospitals in VN.

Methods

Data collection

Data collected at 3-, 5-, 8-months, included focus group discussions and in-depth interviews with patients, peer mentors and healthcare workers.

Data translation

Excerpts were translated into English by experts and each was assigned a unique ID for anonymity.

CFIR coding

Translated data was coded by two independent analysts for CFIR implementation outcomes.

Theme refinement

Research team met weekly to adjudicate codes and refined themes.

CFIR Domain and Construct	Acceptability		Feasibility	
	Barrier	Facilitator	Barrier	Facilitator
Innovation				
Innovation Source				
Innovation Evidence-Base				
Innovation Relative Advantage				
Innovation Adaptability				
Innovation Trialability				
Innovation Complexity				
Innovation Design				
Innovation Cost				
Outer Setting				
Critical Incidents				
Local Attitudes				
Local Conditions				
Partnerships & Connections				
Policies & Laws				
Financing				
External Pressure				
Societal Pressure				
Market Pressure				
Performance-Measurement Pressure				
Process				
Teaming				
Assessing Needs				
Innovation Deliverers				
Innovation Recipients				
Assessing Context				
Planning				
Tailoring Strategies				
Engaging				
Innovation Deliverers				
Innovation Recipients				
Doing				
Reflecting & Evaluating				
Implementation				
Innovation				
Adapting				

CFIR Domain and Construct	Acceptability		Feasibility	
	Barrier	Facilitator	Barrier	Facilitator
Individuals				
-Roles Subdomain:				
A. High-level Leaders				
B. Mid-level Leaders				
C. Opinion Leaders				
D. Implementation Facilitators				
E. Implementation Leads				
F. Implementation Team Members				
G. Other Implementation Support				
H. Innovation Deliverers				
I. Innovation Recipients				
-Characteristics Subdomain:				
A. Need				
B. Capability				
C. Opportunity				
D. Motivation				
Inner Setting				
A. Structural Characteristics				
Physical Infrastructure				
Information Technology Infrastructure				
Work Infrastructure				
B. Relational Connections				
C. Communications				
D. Culture				
Human Equality-Centeredness				
Recipient-Centeredness				
Deliverer-Centeredness				
Learning-Centeredness				
E. Tension for Change				
F. Compatibility				
G. Relative Priority				
H. Incentive Systems				
I. Mission Alignment				
J. Available Resources				
Funding				
Space				
Materials & Equipment				
K. Access to Knowledge & Information				

Results

Meeting critical psychosocial needs of newly diagnosed cancer patients

Major facilitators of both acceptability and feasibility included those in the Innovation and Individual domains, and indicated high satisfaction of the ST program.

Training, supporting peer mentors serves as essential elements

Innovation deliverers and capability highlighted mentors' knowledge and skills improved the program's acceptability and feasibility.

COVID-19 and its impact on implementation feasibility

An outer setting factor, COVID-19, was cited as a major barrier for feasibility. It forced the program to be delivered virtually and likely modulated the intended impact on patients' perceived social support.

Discussion

The high acceptability and feasibility of ST implementation in VN are primarily due to the fit between the individual characteristics and innovation factors in several ways:

1. The intervention design aligned with newly diagnosed patient **unmet needs** in search of informational and emotional support to understand and engage in treatment.
2. **The non-hierarchical, reciprocal relationship with peer mentors** allows patients to express genuine feelings and concerns, especially important in the context of the hierarchical nature of Asian society.
3. The process of **sharing lived experiences** allows mentors to perceive their roles as valuable and personally fulfilling.

- **Limitation:** non-randomized design may give rise to selection bias; study period coincided with COVID-19.

Conclusion

The ST program has strong potential to enhance psychosocial support among cancer patients and provide a source of supportive care requiring minimal resources in low-resource settings.

Reference

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- ² Kiemen A, Czornik M, Weis J. How effective is peer-to-peer support in cancer patients and survivors? A systematic review. J Cancer Res Clin Oncol. 2023;149(11):9461-9485. doi:10.1007/s00432-023-04753-8
- ³ Damschroder LJ, Reardon CM, Widerquist MAO, Lowery J. The updated Consolidated Framework for Implementation Research based on user feedback. Implement Sci. 2022;17(1):75. Published 2022 Oct 29. doi:10.1186/s13012-022-01245-0